



WNHS PROCEDURE	
Admission to the Mother Baby Unit (MBU)	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	WHGMH, MBU

1. Aim

The purpose of this procedure is to ensure all Mother and Baby Unit (MBU) patients are admitted according to the admission criteria and process set by the [Women's and Perinatal Mental Health Referral and Management Guideline](#) and is aligned with evidence-based best practice and service delivery.

2. Background

The MBU aspires to ensure the rights of a woman and partner/support person are met according to [Charter of Mental Health Care Principles](#) (*Mental Health Act (MHA) 2014*).

3. Risk

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☐
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

4. Key Points

Admission criteria to the MBU is set out in the [Women's and Perinatal Mental Health Referral and Management Guideline](#).

5. Process

Prior to admission:

- All admissions to the MBU are triaged as per [Referral to Mother Baby Unit](#) procedure.
- Admissions will be on a planned basis whenever possible.

- Once a patient is accepted for admission, they/their carer/referring service are contacted by the Clinical Nurse Manager/Clinical Nurse Consultant/Shift Coordinator and given a date and time for admission.
- The timing of the admission is arranged to facilitate practical arrangements for the partner/support person (including other children) where possible.
- The patient is provided practical information with regards to what to bring and expect on arrival at the unit. The patient is also informed about limitations on the unit, as required, which may include smoking and co-sleeping.
- Medical responsibility for treatment and management is assigned to one of the Consultant Psychiatrists.
- As required, the admitting nurse must check all relevant legal documents for validity before the woman is accepted. This includes ensuring any forms under the [Mental Health Act 2014](#) are complete and correct.
- If required, an interpreter should be arranged prior to admission.

On arrival at MBU

- A brief health check will be completed to ensure the patient and baby is physically well – temperature, pulse, respirations and blood pressure, and any current health screening questions completed. Although the baby is technically a boarder and not a patient of the unit, the MBU has a responsibility to ensure the baby does not have an infectious illness that requires management, respond to the baby's health care needs if required and to manage any regular medications as per hospital standards. This may require liaison with other health professionals and/or transfer to Perth Children's Hospital if necessary. Hospital admission paperwork is completed.
- The partner/support person is identified in consultation with the patient and contact details are documented in the patient medical record.
- The patient and partner/support person (where available) are introduced to staff, shown around the unit and given a welcome pack which includes a copy of the [Charter of Mental Health Care Principles \(MHA 2014\)](#), and brochures relating to rights and responsibilities, representation, advocacy, complaints procedure, services and access to personal information. Copies are made available to partner/support person.
- The patient and partner/support person are to be given a brief outline of what examinations, assessments and processes will be completed in the first 24 hours after admission.
- All patient's items will be checked to maintain a safe ward environment. Refer to [MBU Search procedure](#).
- Staff will be sensitive to the individual presentation of the patient and manage admission tasks taking this into consideration.

On Admission

- A formal medical assessment and admission
 - This will include mental health assessment by a psychiatrist and a physical health examination by a medical officer.
 - The Mental Health Assessment (SMHMR902) will be completed on webpsolis and includes history of presenting problem, past psychiatric history, legal issues including any current legal orders (VROs or CPFS stipulations), drug and alcohol history, medical history, allergies, current treatments, social history including living situation and family domestic violence, developmental history, mental state examination, formulation and initial management plan. This will include any visiting restrictions.
 - All patients will be reviewed by a psychiatrist within 24 hours of admission.
 - A meeting with the patient's partner or support person should be organised at the time of admission or soon after, to ensure that the family and the MBU team are collaborating effectively in planning for recovery and discharge.
- A risk assessment and management plan is completed on admission and any immediate identified risks are documented in the patient's healthcare record with current risks communicated to all relevant staff. The risk assessment informs the level of observation for mother and baby and is noted in the healthcare record and ward journey board.
- All patients admitted to the Mother and Baby Unit are required to have a falls risk assessment within 6 hours of admission using the North Metropolitan Area Health Service Mental Health Falls Risk Management Tool (FRMT) PMR 100. See [WNHS Procedure Falls: Risk Assessment and Management of Patient Falls](#).
- The level of observation and rationale behind this is discussed with the patient and partner/support person.
- Plans for baby care will be identified with the patient and partner/support person, including a night sleep plan and a feeding plan/preference.
- The admission pack containing Self Report Scales is given to the patient on admission to the unit and must be completed within 72 hours of admission, scored by the Clinical Psychologist within 24 hours of completion (if during work days) and the results recorded in the patient's healthcare record. Nursing staff will be required to check the results of the measures prior to scoring by the Clinical Psychologist.
- A physical examination is to be initiated within 12 hours of admission (see [Physical Health Care of Inpatients procedure](#))

- Mandatory notifications will be made within the required time frames in accordance with the [Mental Health Act 2014](#).
- The nurse completes a HONOS and NOCC assessment and a PSOLIS management plan as soon as possible following admission. A Treatment, Support and Discharge Plan (TSDP) will be commenced to establish goals of admission.
- Following the first review of the patient by the psychiatrist, an approximate discharge date will be given. This is documented on the patient's Treatment, Support & Discharge Plan (TSDP) which is given to the patient and their partner/support person.
- Staff will support patients to complete the Kessler 10 and then enter the results into PSOLIS.

Women of Aboriginal or Torres Strait Islander descent

- Women who are admitted to the MBU of Aboriginal or Torres Strait Islander descent are assessed, examined, treated and provided with care that is appropriate to, and consistent with, their cultural practices.
- Staff must consider the views of the patient's family, and to the extent that is practicable and appropriate to do so, significant members of their communities, including elders and traditional healers, and Aboriginal mental health workers.
- Consultation may include the KEMH Aboriginal Liaison Officer, Wungen Kartup (metropolitan area) and WACHS Specialist Aboriginal Mental Health Service.
- Visits from external community services, as well as wider family, will be facilitated wherever possible to ensure cultural safety.

Women under the age of 18 years

- Patients under 18 years are subject to special provision under the MHA 2014 which must be reflected in their admission, treatment and discharge processes [Applying the MHA in WNHS](#).

Related internal policies, procedures and guidelines

[Charter of Mental Health Care Principles](#)

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

[Applying the MHA in WNHS](#)

[Referral to the Mother Baby Unit](#)

[MBU Search procedure](#)

[Physical Health Care of MBU Inpatients Procedure](#)

References

[Mental Health Act 2014](#)

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Policy Sponsor	Head of Department Mother Baby Unit/ Director of Psychiatry				
Policy Author	Coordinator of Nursing – Mental Health				
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NSQHS Standards Applicable:	 Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 3: Preventing and Controlling Healthcare Associated Infection		 Std 5: Comprehensive Care  Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration		
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Version History:

Version Notes	Dates
First published version	February 2007
Regular review and update	December 2009, October 2011, September 2012, January 2013, September 2013, 16 May 2017, 26 March 2019, September 2019, August 2021
Three yearly review – amendments include: • Hyperlinks fixed. Added: • Limitations on the unit • Health check of baby boarder and infectious illness • More detailed information on Mental Health Assessment • Falls risk assessment • Visits from external community services for cultural safety • Projected discharge plan changed to Treatment Support and Discharge plan • Feeding regime changed to plan/preference	May 2025

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